Drug Policy

The Green Party will use an evidence-based, harm minimisation approach to alcohol and other drug law in place of criminal prohibition on one hand and commercial privileges on the other. We will provide accessible and affordable services for substance use disorders, a legal and regulated cannabis market, and other harm reduction measures, such as education.

Vision

An evidence-based legal framework promotes and delivers health, wellbeing and informed agency where people may choose to use substances.

Values and Principles

Drug policy must be consistent with the following values and principles:

- *Honour Te Tiriti o Waitangi*: Drug law is part of the wider colonial system that has harmed Māori. The Crown has obligations to reverse those harms and actively protect Māori health, wellbeing, and rangatiratanga in drug policy.
- *Ecological Wisdom*: Drug production, trade, use and enforcement must be constrained by the wellbeing limits of the communities and ecosystems that they operate within.
- *Social Responsibility*: Substance-related harm should be treated as a health issue, with a focus on root causes and equity.
- *Appropriate Decision-Making*: Drug policy should reflect current evidence and be tailored to those most affected by problematic drug use. It should enable informed agency where people may choose to use substances.
- *Non-Violence*: Drug policy should have a primary focus on improving wellbeing and reducing harm, not on punishing users.

Strategic Priorities

The Green Party's strategic goals include:

> *"As a Party we strive to create a more connected, compassionate and equal Aotearoa, free from structural biases that discriminate against groups and individuals."*

Actions in this policy that will help achieve this include:

- Ensure that drug-related legislation and regulation effectively reduces harm and cost to society, individuals, and the environment from drug production and use. (1.1.1)
- Implement and sufficiently resource approaches that are found to be the best to reduce the suffering of people with a substance use disorder or other problematic drug use, including services that minimise harm during drug use, such as needle and syringe exchange schemes and safe consumption spaces. (2.2.1)

- Regulate cannabis for legal personal use (...). (3.1)
- Phase out advertising of alcoholic beverages and sponsorship by alcohol brands, and increase funding for sporting organisations to fill the resulting funding gaps. (3.7.1)

Connected Policies

Harmful drug use must be treated first as a Health issue, rather than a criminal Justice issue. Drug use does not occur in a vacuum so, for example, improving Livelihoods, Housing, and Education also help reduce drug-related harm.

Policy Positions

1. Drug Law Reform

Issues

Since the Misuse of Drugs Act became law in 1975, tens of thousands of people have been imprisoned, the global illicit drug supply has become more volatile and toxic, and overdose deaths and chronic drug harm have increased. Māori have been disproportionately affected by drug harm and the criminal justice system. Drug criminalisation leads to unsafe drug use, fosters organised crime, and traps those convicted in a cycle of poverty that, in turn, contributes to harmful drug use.

Actions

1.1. Ensure that drug-related legislation and regulation:

1.1.1. Effectively reduces harm and cost to society, individuals, and the environment from drug production and use;

1.1.2. Enhances people's capacity for informed choice;

1.1.3. Is evidence-based, recognising that:

1.1.3.1. There can be adverse health, social, environmental, and economic consequences from drug use for both individuals and society;

1.1.3.2. Not all drug use is abusive or problematic;

1.1.3.3. Some individuals in society have always chosen to use substances, regardless of their legal status;

1.1.3.4. Substance-related harm disproportionately impacts some groups, including tangata whenua; and

1.1.3.5. Criminal prohibition of substances causes more harm than it prevents; and

1.1.4. Is co-designed with Māori, under a Te Tiriti o Waitangi framework, and promotes the rights and interests of whānau, hapū and iwi.

1.2. As a short-term measure, implement the recommendations of the Law Commission's 2011 review of the Misuse of Drugs Act 1975.

2. Drug harm reduction

Issues

Drug-related harm is significant and increasing in Aotearoa New Zealand, driven mainly by alcohol, methamphetamine, and emerging cocaine use. Dependence on alcohol or other drugs affects a person's physical and mental health, relationships, employment, and personal finances. It can also affect the family and friends of people who use drugs and the victims of drug-related or drug-funded crime. Along with criminal prohibition, the significant stigma associated with substance dependence and a long-standing lack of harm reduction services mean that a large proportion of people with substance use disorders want help but can't get it.

Actions

2.1. Ensure that evidence-based, non-judgemental harm-reduction information is available to those at risk of harmful alcohol and other substance use, including by:

2.1.1. Providing age-appropriate education about substance use disorders in schools; and

2.1.2. Providing accessible information about substance use disorders, through health services and at the point of sale.

2.2. Implementing and sufficiently resourcing approaches that are found to be the best to reduce the suffering of people with a substance use disorder or other problematic drug use, including:

2.2.1. Services that minimise harm during drug use, such as needle and syringe exchange schemes and safe consumption spaces;

2.2.2. Cessation programmes, such as smoking cessation programmes for people with tobacco dependence;

2.2.3. Regulated, medically-supervised supply to people experiencing substance dependence, with wrap-around support;

2.2.4. Substitution programmes;

2.2.5. Substance use disorder programmes that are integrated within health and social services systems;

2.2.6. Targeted, accessible and culturally responsive alcohol and other drug rehabilitation services, with long-term support; and

2.2.7. Innovative and Tiriti-based programmes that provide help rather than punishment for people with drug problems.

2.3. Strengthen, diversify and resource substance-harm treatment programmes, including by:

2.3.1. Providing culturally safe treatment programmes in underserved communities; and

2.3.2. Providing accessible, prompt, effective overdose treatments.

3. Substance-specific initiatives

Issues

Aotearoa New Zealand has a harmful and problematic drinking culture. Heavy alcohol use is related to fatal car crashes, mental health crises, and many other negative impacts that cost the country more than $7bn each year. Cannabis causes far less harm than alcohol, methamphetamine, cocaine, or tobacco, ranking lowest among common illicit drugs.

Actions

A. Cannabis

3.1. Regulate cannabis for legal personal use, such that:

- 3.1.1. Te Tiriti o Waitangi is upheld;
- 3.1.2. Possession and personal use of cannabis and/or cannabis products is no longer illegal and previous criminal convictions for these offences are removed from all records;
- 3.1.3. Cultivation for personal use is no longer illegal (within legislated limits);
- 3.1.4. Evidence from overseas jurisdictions with legal cannabis markets is assessed to determine which regulatory model best minimises harm; and
- 3.1.5. A legal age limit for personal cannabis use is introduced.

3.2. Minimise harm from cannabis use by:

- 3.2.1. Improving drug-driving laws to ensure that they reflect current evidence and effectively deter driving while impaired while upholding human rights;
- 3.2.2. Providing education on the harmful effects of heavy and prolonged cannabis usage and particularly vulnerable groups;
- 3.2.3. Providing support to address harmful consequences of heavy and prolonged cannabis usage; and
- 3.2.4. Including cannabis in the provisions of the Smokefree Environments Act.

3.3. Improve the regulatory framework for medicinal cannabis to ensure accessibility and affordability for patients, including by:

- 3.3.1. Removing penalties for any person with a medically-determined and approved reason to cultivate, possess or use cannabis and/or cannabis products for therapeutic purposes.

3.4. Accelerate the process by which medical cannabis products are licensed and made available, including by:

- 3.4.1. Establishing category-based classes for common compositions of medical cannabis products; and
- 3.4.2. Removing unnecessary barriers for manufacturers to submit new cannabis products for funding applications to PHARMAC to accelerate information sharing.

3.5. Ensure that new and approved cannabis-based medicines meet acceptable standards of safety, quality and efficacy, by:

- 3.5.1. Encouraging MedSafe to carry out extensive ongoing monitoring; and

3.5.2. Providing health practitioners with reliable information about the selection and safe use of these products when prescribing them.

B. Alcohol

3.6. Minimise harm from alcohol use by:

3.6.1. Strengthening law enforcement on the illegal sale and supply of alcohol to minors; and

3.6.2. Requiring accurate health information to be displayed on all alcohol products, including evidence-based health warnings on alcohol containers.

3.7. Reduce normalisation of alcohol use by:

3.7.1. Phasing out advertising of alcoholic beverages and sponsorship by alcohol brands, and increasing funding for sporting organisations to fill the resulting funding gaps; and

3.7.2. Requiring alcohol retailers to minimise the overt display of alcohol.

3.8. Disincentivise alcohol consumption by:

3.8.1. Setting a minimum price per standard drink for alcohol;

3.8.2. Adopting an alcohol tax that is directly proportional to the quantity of pure alcohol in a drink; and

3.8.3. Increasing alcohol tax levels to reflect the costs incurred through harmful alcohol use.

3.9. Support the establishment and continuation of licensing trusts that encourage responsible attitudes to alcohol in their mission statement.

C. Other substances

3.10. Achieve the Smokefree Aotearoa goal, including by:

3.10.1. Extending the Smokefree Environments Act; and

3.10.2. Funding innovative approaches to working with groups where smoking rates are disproportionately high, such as for Tagata Moana and Māori.

3.11. Establish mandatory regulatory systems for all novel recreational psychoactive substances, and monitor any harm caused by their availability.

3.12. Ensure vaping regulation restricts youth uptake through evidence-based policy.